

Diagnostic Audit

Scope of Work · Haven Paediatric Centre

Workstream 1 of the Consult for Africa engagement | Duration 4 weeks | Fee N2,100,000 (mobilisation, on signing)

Objective

To establish, from evidence rather than impression, exactly where Haven stands across clinical safety, culture, operations, and cash, and to leave the board with a prioritised, costed plan of action. The audit is the foundation that shapes every workstream that follows. Two safety and cash quick wins are implemented during the audit, not after it.

What we will examine

- **Clinical governance and patient safety.** The incident and near-miss reporting system, crash-cart and emergency readiness, clinical protocols, NICU readiness, and the shift-level safety routines that underpin reliable care.
- **Culture, routines, and incentives.** How work actually gets done, where nursing and operational routines break down, and whether the commission structure, JDS and KPIs reward quality and ownership.
- **Operations and SOP adherence.** Patient flow, customer service and booking, and the gaps between documented SOPs and what happens.
- **Finance and working capital.** Revenue mix and yield (private versus HMO), margins, the ~N4.2M in HMO receivables, the ~N2.77M in pharmacy stock, and where cash is locked.
- **Procurement and inventory.** Stock management, procurement reliability, pharmacy margin, and the case for a vendor-managed inventory model.
- **HMO economics.** Contract terms, claims and ageing, and yield per HMO versus private.
- **People and structure.** Staffing ratio, roles, and the specification for the senior operations leader role.
- **Management reporting integrity.** Whether the numbers the board sees can be trusted, starting with the reconciliation gaps in the current report.

How we will work

Structured interviews with leadership and frontline staff (nursing, pharmacy, customer service, operations), direct observation on site, review of financial, clinical and operational records, and a short data request issued in week one. The approach is collaborative, not audit-by-ambush. The team is a partner in the findings.

Quick wins delivered during the audit

- A stocked, sealed, checklist-governed crash cart with a shift-level check routine, live in week one.
- An immediate recovery push on the largest HMO receivables (Leadway and NEM), so cash starts moving before the audit even reports.

Deliverables

- **Operational and Clinical Governance Audit Report.** Findings across every area above, with prioritised, costed recommendations.
- **Working Capital and Receivables Recovery Plan.** A concrete plan to unlock the cash tied up in receivables and stock.

Both are presented to the board in a working session at the end of week four.

Timeline

Week	Focus
Week 1	Mobilise, interviews, data request, crash-cart standard live, receivables push begins
Week 2	Clinical governance and safety, culture and routines, operations and SOP walkthrough
Week 3	Finance and working capital, procurement and inventory, HMO economics, reporting integrity
Week 4	Synthesis, prioritised recommendations, board presentation

What we will need from Haven

- A named point person for the engagement (we suggest the Head of Operations).
- Read access to financial records, pharmacy stock data, HMO contracts, the staff roster and job descriptions, clinical incident records, and existing SOPs.
- Time with leadership and the nursing, pharmacy, and customer-service leads.
- Walk-through access to the facility.

Fee and next step

The diagnostic audit is the mobilisation phase of the engagement: **N2,100,000, payable on signing** (standard rate N3,500,000, at the 40 percent partner discount). On the board's go-ahead, we mobilise within the week.

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